

**NAME OF THE PRODUCT:** Ezetimibe Tablets 10 mg  
**(TRADE) NAME OF PRODUCT:** ETIMICOL 10  
**STRENGTH:** 10 mg  
**PHARMACEUTICAL DOSAGE FORM:** Tablet  
**QUALITATIVE AND QUANTITATIVE COMPOSITIONS:**  
*Ezetimibe Tablets 10 mg*  
Each tablet contains 10 mg of Ezetimibe

**PHARMACEUTICAL FORM**  
*Ezetimibe Tablets 10 mg:* White to off-white uncoated, capsule shaped bevelled edge tablets debossed with 'E Z' on one side and '10' on other side.

**CLINICAL PARTICULARS**  
**Therapeutic indications**  
Primary Hypercholesterolaemia  
Ezetimibe tablets administered alone, or with an HMG-CoA reductase inhibitor (statin), is indicated as adjunctive therapy to diet in patients with primary (heterozygous familial and non-familial) hypercholesterolaemia.

Ezetimibe tablets, administered in combination with fenofibrate, is indicated as adjunctive therapy to diet for the reduction of elevated total-C, LDL-C, Apo B, and non-HDL-C in patients with mixed hyperlipidemia.

**Prevention of Cardiovascular Events**  
Ezetimibe tablets, administered with a statin, is indicated to reduce the risk of cardiovascular events (cardiovascular death, nonfatal myocardial infarction, nonfatal stroke, hospitalization for unstable angina, or need for revascularization), in patients with coronary heart disease (CHD) and a history of acute coronary syndrome (ACS).

Homozygous Familial Hypercholesterolaemia (HoFH)  
Ezetimibe tablets, administered with a statin, is indicated for patients with HoFH. Patients may also receive adjunctive treatments (e.g., LDL apheresis).

Homozygous Sitosterolaemia (Phytosterolaemia)  
Ezetimibe tablets is indicated for the reduction of elevated sitosterol and campesterol levels in patients with homozygous familial sitosterolaemia.

**Posology and method of administration**  
**Posology**  
The patient should be on an appropriate lipid-lowering diet and should continue on this diet during treatment with Ezetimibe Tablets. Route of administration is Oral.

**Use in Patients with Primary Hypercholesterolemia**  
The recommended dose of ezetimibe tablets is 10 mg once daily, used alone, with a statin or with fenofibrate. Ezetimibe tablets can be administered at any time of the day, with or without food.

Ezetimibe tablets may be administered with a statin (in patients with primary hypercholesterolemia) or with fenofibrate (in patients with mixed hyperlipidemia) for incremental effect. For convenience, the daily dose of ezetimibe tablets may be taken at the same time as the statin or fenofibrate, according to the dosing recommendations for the respective medications.

If ezetimibe 10mg tablets are used in combination with a statin therapy, the dosage instructions for that particular statin should be consulted.

When initiating lipid lowering treatment, which includes ezetimibe 10mg tablets and a statin in combination, the indicated usual initial dose of that particular statin should be used or the already established higher statin dose should be continued.

If the statin dose is to be increased for the first time or further, the dosage instructions of that particular statin should be followed (such as dose increase only after at least 4 weeks of regular use of the combination without any change).

The stepwise increase of the statin dose in combination treatment results in a relatively small additional decrease of LDL-C, but increases the risk of dose-related adverse events of the statin. This has to be considered for the risk-benefit-assessment when the statin dose is considered.

**Use in Patients with Coronary Heart Disease**  
Combination Therapy with a Statin  
For incremental cardiovascular event reduction in patients with coronary heart disease, ezetimibe 10 mg may be administered with a statin with proven cardiovascular benefit.

Use in the Elderly  
No dosage adjustment is required for elderly patients. However, greater sensitivity of some older individuals cannot be ruled out.

Use in Paediatric Patients  
Children and adolescents ≥ 10 years: No dosage adjustment is required.  
Children < 10 years: Treatment with ETIMICOL is not recommended.

Use in Hepatic Impairment  
No dosage adjustment is required in patients with mild hepatic insufficiency (Child Pugh score 5 to 6). Treatment with ezetimibe tablets is not recommended in patients with moderate (Child Pugh score 7 to 9) or severe (Child Pugh score > 9) liver dysfunction.

Use in Renal Impairment  
No dosage adjustment is required for renally impaired patients.

Co-administration with bile acid sequestrants  
Dosing of ezetimibe tablets should occur either ≥ 2 hours before or ≥ 4 hours after administration of a bile acid sequestrant.

**Contraindications**  
Hypersensitivity to the active substance or to any of the excipients used in the manufacture of Ezetimibe tablets.

When Ezetimibe Tablets is co-administered with a statin, please refer to the SPC for that particular medicinal product.

Therapy with Ezetimibe Tablets co-administered with a statin is contraindicated during pregnancy and lactation.

Ezetimibe Tablets co-administered with a statin is contraindicated in patients with active liver disease or unexplained persistent elevations in serum transaminases.

**Special warnings and precautions for use**  
When Ezetimibe Tablets is co-administered with a statin, please refer to the SPC for that particular medicinal product.

Liver Enzymes  
Patients receiving Ezetimibe Tablets with a statin, consecutive transaminase elevations (≥ 3 X the upper limit of normal [ULN]) may be observed. When Ezetimibe Tablets is co-administered with a statin, liver function tests should be performed at initiation of therapy and according to the recommendations of the statin.

In patients with coronary heart disease and ACS event history, the incidence of consecutive elevations of transaminases (≥3 X ULN) may be more in patients receiving ezetimibe/simvastatin than patients receiving simvastatin.

Skeletal Muscle  
Patients receiving Ezetimibe Tablets, cases of myopathy and rhabdomyolysis may be observed. Most patients who develop rhabdomyolysis will be taking a statin concomitantly with Ezetimibe Tablets. However, rhabdomyolysis may be reported very rarely with

Ezetimibe Tablets monotherapy and very rarely with the addition of Ezetimibe Tablets to other agents known to be associated with increased risk of rhabdomyolysis. If myopathy is suspected based on muscle symptoms or is confirmed by a creatine phosphokinase (CPK) level >10 times the ULN, Ezetimibe Tablets, any statin, and any of these other agents that the patient is taking concomitantly should be immediately discontinued. All patients starting therapy with Ezetimibe Tablets should be advised of the risk of myopathy and told to report promptly any unexplained muscle pain, tenderness or weakness.

In patients with coronary heart disease and ACS event history, the incidence of myopathy may be more in patients receiving ezetimibe/simvastatin than patients receiving simvastatin. The incidence of rhabdomyolysis may be less in patients receiving ezetimibe/simvastatin than patients receiving simvastatin.

Patients with hepatic impairment  
Due to the unknown effects of the increased exposure to ezetimibe in patients with moderate or severe hepatic impairment, Ezetimibe Tablets is not recommended.

Paediatric population  
Existing data has demonstrated the efficacy and safety of Ezetimibe in patients 6 to 10 years with heterozygous familial or non-familial hypercholesterolemia in a duration of 12 weeks or less. However, there is no data to for treatment periods in this age group for longer than 12 weeks.

Ezetimibe has not been studied in patients younger than 6 years of age.

Based on existing data demonstrating efficacy and safety of Ezetimibe co administered with Simvastatin in patients 10 to 17 years of age with heterozygous familial hypercholesterolaemia (HeFH) in adolescent boys (Tanner stage II or above) and in girls who were at least 1-year post menarche, it was found that:

There was generally no detectable effect on growth or sexual maturation in the adolescent boys or girls, or any effect on menstrual cycle length in girls.

However, the effects of Ezetimibe for a treatment period > 33 weeks on growth and sexual maturation have not been studied.

The safety and efficacy of Ezetimibe co administered with doses of Simvastatin above 40 mg daily have not been studied in paediatric patients 10 to 17 years of age.

The safety and efficacy of Ezetimibe co administered with Simvastatin have not been studied in paediatric patients < 10 years of age.

The long term efficacy of therapy with Ezetimibe in patients below 17 years of age to reduce morbidity and mortality in adulthood has not been studied.

Fibrates  
The safety and efficacy of Ezetimibe Tablets administered with fibrates have not been established. If cholelithiasis is suspected in a patient receiving Ezetimibe Tablets and fenofibrate, gallbladder investigations are indicated and this therapy should be discontinued.

Ciclosporin  
Caution should be exercised when initiating Ezetimibe Tablets in the setting of ciclosporin. Ciclosporin concentrations should be monitored in patients receiving Ezetimibe Tablets and ciclosporin.

Anticoagulants  
If Ezetimibe Tablets is added to warfarin, another coumarin anticoagulant, or fludione, the International Normalised Ratio (INR) should be appropriately monitored.

Excipient  
Patients with rare hereditary problems of galactose intolerance, the Lapp lactase deficiency or glucose-galactose malabsorption should not take this medicine.

**Interaction with other medicinal products and other forms of interaction**  
Ezetimibe does not induce cytochrome P450 drug metabolising enzymes.

No clinically significant pharmacokinetic interactions have been observed between ezetimibe and drugs known to be metabolised by cytochromes P450 1A2, 2D6, 2C8, 2C9, and 3A4, or N-acetyltransferase.

Ezetimibe had no effect on the pharmacokinetics of dapsone, dextromethorphan, digoxin, oral contraceptives (ethinyl estradiol and levonorgestrel), glipizide, tolbutamide, or midazolam during co-administration.

Cimetidine, co-administered with ezetimibe, had no effect on the bioavailability of ezetimibe.

Antacids  
Concomitant antacid administration decreased the rate of absorption of ezetimibe but had no effect on the bioavailability of ezetimibe. This decreased rate of absorption is not considered clinically significant.

Cholestyramine  
Concomitant cholestyramine administration decreased the mean area under the curve (AUC) of total ezetimibe (ezetimibe + ezetimibe glucuronide) approximately 55 %. The incremental low-density lipoprotein cholesterol (LDL-C) reduction due to adding Ezetimibe Tablets to cholestyramine may be lessened by this interaction.

Fibrates  
In patients receiving fenofibrate and Ezetimibe Tablets, physicians should be aware of the possible risk of cholelithiasis and gallbladder disease.

If cholelithiasis is suspected in a patient receiving Ezetimibe Tablets and fenofibrate, gallbladder investigations are indicated and this therapy should be discontinued.

Concomitant fenofibrate or gemfibrozil administration modestly increased total ezetimibe concentrations (approximately 1.5- and 1.7-fold respectively).

Co-administration of Ezetimibe Tablets with other fibrates has not been studied.

Fibrates may increase cholesterol excretion into the bile, leading to cholelithiasis. In animal studies, ezetimibe sometimes increased cholesterol in the gallbladder bile but not in all species. A lithogenic risk associated with the therapeutic use of Ezetimibe Tablets cannot be ruled out.

Statins  
No clinically significant pharmacokinetic interactions were seen when ezetimibe was co-administered with atorvastatin, simvastatin, pravastatin, lovastatin, fluvastatin, or rosuvastatin.

Ciclosporin  
Caution should be exercised when initiating Ezetimibe Tablets in the setting of ciclosporin. Ciclosporin concentrations should be monitored in patients receiving Ezetimibe Tablets and ciclosporin.

Anticoagulants  
If Ezetimibe Tablets is added to warfarin, another coumarin anticoagulant, or fludione, INR should be appropriately monitored.  
Paediatric population  
Interaction studies have only been performed in adults.

**Fertility, pregnancy and lactation**  
Ezetimibe Tablets co-administered with a statin is contraindicated during pregnancy and lactation, please refer to the SPC for that particular statin.

Pregnancy  
Ezetimibe Tablets should be given to pregnant women only if clearly necessary. No clinical data are available on the use of Ezetimibe Tablets during pregnancy.

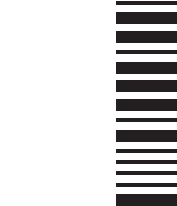
Lactation  
Ezetimibe Tablets should not be used during lactation.

Fertility  
No clinical trial data are available on the effects of ezetimibe on human fertility.

P1523456

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|------------------------------------------------------------------------------------------------------------------------------------------------------|--------------------------------------------|----------------------------------------------------------|------------------------------------------------------------------------------------------------------------------|--------------------------------------------------|-----------------------------------------------------------------|
| <div></div> <div>AUROBINDO</div> <div>Packaging Development</div> |                                            | <div>Product Name</div> <div>Etimicol 10 mg</div>        | <div>Component</div> <div>Leaflet</div>                                                                          | <div>Item Code</div> <div>P1523456</div>         | <div>Date &amp; Time</div> <div>28.04.2021 &amp; 10.00 am</div> |
|                                                                                                                                                      |                                            | <div>Customer / Country</div> <div>Malaysia_Unit-7</div> | <div>Version No.</div> <div>05</div>                                                                             | <div>Reason Of Issue</div> <div>Submission</div> | <div>Reviewed / Approved by</div>                               |
| <div>Team Leader</div> <div>Kiran Kumar</div>                                                                                                        | <div>Initiator</div> <div>Shirisha N</div> | <div>Dimensions</div> <div>140 x 360 mm</div>            | <div>No. of Colours : 01</div>                                                                                   |                                                  |                                                                 |
| <div>Artist: SCDesigners</div>                                                                                                                       |                                            | <div>Pharmacode</div> <div>23456</div>                   | <div></div> <div>23456</div> |                                                  |                                                                 |
| <div>Additional Information :<br/>Word Replacement :28.04.2021</div>                                                                                 |                                            |                                                          |                                                                                                                  |                                                  |                                                                 |
|                                                                                                                                                      |                                            |                                                          |                                                                                                                  |                                                  | <div>Sign / Date</div>                                          |



Effects on ability to drive and use machines

No studies on the effects on the ability to drive and use machines have been performed. However, when driving vehicles or operating machines, it should be taken into account that dizziness may be reported.

Undesirable effects

Side effects on the system organ class alongside its frequencies are tabulated as follows:

Ezetimibe Tablets monotherapy

| System organ class                                                                | Adverse reactions                                                                                                | Frequency          |
|-----------------------------------------------------------------------------------|------------------------------------------------------------------------------------------------------------------|--------------------|
| Investigations                                                                    | ALT and/or AST increased; blood CPK increased; gamma-glutamyltransferase increased; liver function test abnormal | Uncommon           |
| Respiratory, Thoracic and Mediastinal Disorders                                   | cough                                                                                                            | Uncommon           |
| Gastrointestinal Disorders                                                        | abdominal pain; diarrhoea; flatulence                                                                            | Common             |
|                                                                                   | dyspepsia; gastrooesophageal reflux disease; nausea                                                              | Uncommon           |
| Musculoskeletal And Connective Tissue Disorders                                   | arthralgia; muscle spasms; neck pain                                                                             | Uncommon           |
| Metabolism and Nutrition Disorders                                                | decreased appetite                                                                                               | Uncommon           |
| Vascular Disorders                                                                | hot flush; hypertension                                                                                          | Uncommon           |
| General Disorders And Administration Site Condition                               | fatigue                                                                                                          | Common             |
|                                                                                   | chest pain, pain                                                                                                 | Uncommon           |
| Additional adverse reactions with Ezetimibe Tablets co-administered with a statin |                                                                                                                  |                    |
| System organ class                                                                | Adverse reactions                                                                                                | Frequency          |
| Investigations                                                                    | ALT and/or AST increased                                                                                         | Common             |
| Nervous System Disorders                                                          | headache                                                                                                         | Common             |
|                                                                                   | paraesthesia                                                                                                     | Uncommon           |
| Gastrointestinal Disorders                                                        | dry mouth; gastritis                                                                                             | Uncommon           |
| Skin And Subcutaneous Tissue Disorders                                            | pruritus; rash; urticaria                                                                                        | Uncommon           |
| Musculoskeletal And Connective Tissue Disorders                                   | myalgia                                                                                                          | Common             |
|                                                                                   | back pain; muscular weakness; pain in extremity                                                                  | Uncommon           |
| General Disorders And Administration Site Condition                               | asthenia; oedema peripheral                                                                                      | Uncommon           |
| Post-marketing Experience (with or without a statin)                              |                                                                                                                  |                    |
| System organ class                                                                | Adverse reactions                                                                                                | System organ class |
| Blood and lymphatic system disorders                                              | thrombocytopaenia                                                                                                | Not known          |
| Nervous system disorders:                                                         | dizziness; paraesthesia                                                                                          | Not known          |
| Respiratory, thoracic and mediastinal disorders                                   | dyspnoea                                                                                                         | Not known          |
| Gastrointestinal disorders                                                        | pancreatitis; constipation                                                                                       | Not known          |
| Skin and subcutaneous tissue disorders                                            | erythema multiform                                                                                               | Not known          |
| Musculoskeletal and connective tissue disorder                                    | myalgia; myopathy/ rhabdomyolysis                                                                                | Not known          |
| General disorders and administration site conditions                              | asthenia                                                                                                         | Not known          |
| Immune system disorders                                                           | hypersensitivity, including rash, urticaria, anaphylaxis and angiooedema                                         | Not known          |
| Hepatobiliary disorders                                                           | hepatitis; cholelithiasis; cholecystitis                                                                         | Not known          |
| Psychiatric disorders                                                             | depression                                                                                                       | Not known          |

Ezetimibe Tablets co-administered with fenofibrate  
Gastrointestinal disorders: abdominal pain (common)

Paediatric (6 to 17 years of age) Patients

Paediatric (6 to 10 years of age) patients with heterozygous familial or non-familial hypercholesterolaemia, elevations of ALT and/or AST may be observed in ezetimibe/simvastatin patients. No cases of myopathy may be observed.

Adolescent (10 to 17 years of age) patients with heterozygous familial hypercholesterolaemia, elevations of ALT and/or AST may be observed in ezetimibe/simvastatin patients compared to simvastatin monotherapy group. No cases of myopathy may be observed.

Patients with Coronary Heart Disease and ACS Event History

The incidence of myopathy may be more in patients treated with ezetimibe/simvastatin than with simvastatin, where myopathy was defined as unexplained muscle weakness or pain with a serum CK ≥10 times ULN or two consecutive observations of CK ≥5 and <10 times ULN. The incidence of rhabdomyolysis may be less in patients treated with ezetimibe/simvastatin than with simvastatin, where rhabdomyolysis was defined as unexplained muscle weakness or pain with a serum CK ≥10 times ULN with evidence of renal injury, ≥5 times ULN and <10 times ULN on two consecutive occasions with evidence of renal injury or CK ≥10,000 IU/L without evidence of renal injury. The incidence of consecutive elevations of transaminases (≥3 X ULN) may be more in patients treated with ezetimibe/simvastatin than with simvastatin. Gallbladder-related adverse effects may be less in patients treated with ezetimibe/ simvastatin than with simvastatin.

Patients with Chronic Kidney Disease

Myopathy/rhabdomyolysis, Consecutive elevations of transaminases (> 3X ULN) may be observed in patients treated with Ezetimibe Tablets combined with simvastatin.

Overdose

In the event of an overdose, symptomatic and supportive measures should be employed.

PHARMACOLOGICAL PROPERTIES

Pharmacodynamic properties

Pharmacotherapeutic group: Other lipid modifying agents, ATC code: C10A X09

Mechanism of action

Ezetimibe Tablets is in a new class of lipid-lowering compounds that selectively inhibit the intestinal absorption of cholesterol and related plant sterols. Ezetimibe Tablets is orally active and has a mechanism of action that differs from other classes of cholesterol reducing compounds (e.g., statins, bile acid sequestrants [resins], fibric acid derivatives, and plant stanols). The molecular target of ezetimibe is the sterol transporter, Niemann-Pick C1-Like 1 (NPC1L1), which is responsible for the intestinal uptake of cholesterol and phytosterols.

Ezetimibe localises at the brush border of the small intestine and inhibits the absorption of cholesterol, leading to a decrease in the delivery of intestinal cholesterol to the liver; statins reduce cholesterol synthesis in the liver and together these distinct mechanisms provide complementary cholesterol reduction.

Pharmacodynamic effects

Ezetimibe inhibits the absorption of [14C]-cholesterol with no effect on the absorption of triglycerides, fatty acids, bile acids, progesterone, ethinyl estradiol, or fat soluble vitamins A and D. Administration of Ezetimibe Tablets with a statin is effective in reducing the risk of cardiovascular events in patients with coronary heart disease and ACS event history.

Clinical efficacy and safety

Ezetimibe Tablets either as monotherapy or co-administered with a statin significantly reduces total cholesterol (total-C), low-density lipoprotein cholesterol (LDL-C), apolipoprotein B (Apo B), and triglycerides (TG) and increased high-density lipoprotein cholesterol (HDL-C) in patients with hypercholesterolaemia.

Pharmacokinetic properties

Absorption

After oral administration, ezetimibe is rapidly absorbed and extensively conjugated to a pharmacologically active phenolic glucuronide (ezetimibe-glucuronide). Mean maximum plasma concentrations (Cmax) occur within 1 to 2 hours for ezetimibe-glucuronide and 4 to 12 hours for ezetimibe. The absolute bioavailability of ezetimibe cannot be determined as the compound is virtually insoluble in aqueous media suitable for injection. Concomitant food administration (high fat or non-fat meals) had no effect on the oral bioavailability of ezetimibe when administered as Ezetimibe Tablets 10-mg tablets. Ezetimibe Tablets can be administered with or without food.

Distribution

Ezetimibe and ezetimibe-glucuronide are bound 99.7 % and 88 to 92 % to human plasma proteins, respectively.

Biotransformation

Ezetimibe is metabolised primarily in the small intestine and liver via glucuronide conjugation (a phase II reaction) with subsequent biliary excretion. Minimal oxidative metabolism (a phase I reaction) may be observed. Ezetimibe and ezetimibe-glucuronide are the major drug-derived compounds detected in plasma, constituting approximately 10 to 20 % and 80 to 90 % of the total drug in plasma, respectively. Both ezetimibe and ezetimibeglucuronide are slowly eliminated from plasma with evidence of significant enterohepatic recycling. The half-life for ezetimibe and ezetimibe-glucuronide is approximately 22 hours.

Elimination

Following oral administration of 14C-ezetimibe (20 mg) to human subjects, total ezetimibe accounted for approximately 93 % of the total radioactivity in plasma. Approximately 78 % and 11 % of the administered radioactivity were recovered in the faeces and urine, respectively, over a 10-day collection period. After 48 hours, there were no detectable levels of radioactivity in the plasma.

Special Populations

Paediatric population

The pharmacokinetics of ezetimibe are similar between children ≥6 years and adults. Pharmacokinetic data in the paediatric population < 6 years of age are not available. Clinical experience in paediatric and adolescent patients includes patients with HoFH, HeFH, or sitosterolaemia.

Older people

Plasma concentrations for total ezetimibe are about 2-fold higher in the elderly (≥ 65 years) than in the young (18 to 45 years). LDL-C reduction and safety profile are comparable between elderly and young subjects treated with Ezetimibe Tablets. Therefore, no dosage adjustment is necessary in the elderly.

Hepatic impairment

No dosage adjustment is necessary for patients with mild hepatic impairment. Due to the unknown effects of the increased exposure to ezetimibe in patients with moderate or severe (Child-Pugh score > 9) hepatic impairment, Ezetimibe Tablets is not recommended in these patients.

Renal impairment

No dosage adjustment is necessary for renally impaired patients. Patient with post-renal transplant and receiving multiple medications, including ciclosporin may have 12-fold greater exposure to total ezetimibe.

Gender

Plasma concentrations for total ezetimibe are slightly higher (approximately 20 %) in women than in men. LDL-C reduction and safety profile are comparable between men and women treated with Ezetimibe Tablets. Therefore, no dosage adjustment is necessary on the basis of gender.

PHARMACEUTICAL PARTICULARS

List of excipients

Lactose monohydrate, Hypromellose, Croscarmellose sodium, Sodium Lauryl Sulfate, Crospovidone, Cellulose Microcrystalline, Magnesium stearate.

Incompatibilities

Not applicable

Shelf life

Please refer outer package

Special precautions for storage

Do not store above 30°C. Protect from moisture. Keep out of the reach of children.

Nature and contents of container

Blister pack: 3x10's blister pack  
Ezetimibe Tablets 10 mg: Alu-Alu Blister of 10 Tablets.

Manufacturer



AUROBINDO

Aurobindo Pharma Limited (Unit VII)  
Special Economic Zone, TSILC, Plot No. S1, Sy. Nos., 411/P, 425/P, 434/P, 435/P, and 458/P, Green Industrial Park, Polepally Village, Jedcherla Mandal, Mahaboobnagar District, Telangana State, 509302, INDIA

Product Registration Holder in Malaysia:

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DATE OF PREPARATION OF THIS LEAFLET

May 2021